

**PATIENT NAME : BHAVYA SHARMA****REF. DOCTOR : SELF****CODE/NAME & ADDRESS : C000125533**
 AGILUS DIAGNOSTICS PSC-MOHALI (HOME
 SCO NO-2, PHASE-6 S.A.S. NAGAR,RUPNAGAR
 MOHALI 160055
 9872331981
ACCESSION NO : 0080ZE000881**PATIENT ID : BHAVF04100027****CLNT.PATIENT ID: BHAVF04100027****ABHA NO :****AGE/SEX : 25 Years Female****DRAWN : 04/05/2026 09:32:26****RECEIVED : 04/05/2026 11:13:39****REPORTED : 04/05/2026 13:38:49**

Test Report Status	Preliminary	Results	Biological Reference Interval	Units
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HAEMATOLOGY - CBC**COMPLETE CARE ACTIVE WOMEN****BLOOD COUNTS, EDTA WHOLE BLOOD**

HEMOGLOBIN (HB)	13.0	12.0 - 15.0	g/dL
METHOD : CYANMETHEMOGLOBIN METHOD			
RED BLOOD CELL (RBC) COUNT	4.10	3.8 - 4.8	mil/ μ L
METHOD : ELECTRICAL IMPEDANCE			
WHITE BLOOD CELL (WBC) COUNT	9.03	4 - 10	thou/ μ L
METHOD : ELECTRICAL IMPEDANCE			
PLATELET COUNT	215	150 - 410	thou/ μ L
METHOD : ELECTRICAL IMPEDANCE			

RBC AND PLATELET INDICES

HEMATOCRIT (PCV)	39.5	36.0 - 46.0	%
METHOD : ELECTRICAL IMPEDANCE			
MEAN CORPUSCULAR VOLUME (MCV)	96.3	83.0 - 101.0	fL
METHOD : CALCULATED PARAMETER			
MEAN CORPUSCULAR HEMOGLOBIN (MCH)	31.8	27.0 - 32.0	pg
METHOD : CALCULATED PARAMETER			
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION(MCHC)	33.0	31.5 - 34.5	g/dL
METHOD : CALCULATED PARAMETER			
RED CELL DISTRIBUTION WIDTH (RDW)	13.5	11.6 - 14.0	%
METHOD : CALCULATED PARAMETER			
MEAN PLATELET VOLUME (MPV)	11.7 High	6.8 - 10.9	fL
METHOD : CALCULATED PARAMETER			

WBC DIFFERENTIAL COUNT

NEUTROPHILS	55	40 - 80	%
METHOD : LIGHT ABSORBANCE OF CYTCHEMICAL STAINED CELLS IMPEDENCE			
LYMPHOCYTES	36	20 - 40	%
METHOD : LIGHT ABSORBANCE OF CYTCHEMICAL STAINED CELLS IMPEDENCE			
MONOCYTES	8	2.0 - 10.0	%
METHOD : LIGHT ABSORBANCE OF CYTCHEMICAL STAINED CELLS IMPEDENCE			

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Dr.Pranjali Vasishth
 LAB HEAD

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ULR No.775000017592135-0080

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EOSINOPHILS

1

1 - 6

%

BASOPHILS

0

0 - 2

%

METHOD : LIGHT ABSORBANCE OF CYTCHEMICAL STAINED CELLS IMPEDENCE

ABSOLUTE NEUTROPHIL COUNT

4.97

2 - 7

thou/ μ L

METHOD : CALCULATED

ABSOLUTE LYMPHOCYTE COUNT

3.25 High

1.0 - 3.0

thou/ μ L

METHOD : CALCULATED

ABSOLUTE MONOCYTE COUNT

0.72

0.20 - 1.00

thou/ μ L

METHOD : CALCULATED

ABSOLUTE EOSINOPHIL COUNT

0.11

0.02 - 0.50

thou/ μ L

ABSOLUTE BASOPHIL COUNT

0.00

0.0 - 0.10

thou/ μ L

METHOD : CALCULATED

Interpretation(s)

RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.

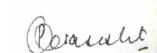
WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease. (Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al.

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This ratio element is a calculated parameter and out of NABL scope.



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HAEMATOLOGY

COMPLETE CARE ACTIVE WOMEN

GLYCOSYLATED HEMOGLOBIN(HBA1C), EDTA WHOLE BLOOD

HBA1C	5.1	Non-diabetic: < 5.7 Pre-diabetics: 5.7 - 6.4 Diabetics: > or = 6.5 ADA Target: 7.0 Action suggested: > 8.0	%
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METHOD : HPLC

ESTIMATED AVERAGE GLUCOSE(EAG)	99.7	< 116	mg/dL
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METHOD : CALCULATED PARAMETER

Interpretation(s)

GLYCOSYLATED HEMOGLOBIN(HBA1C), EDTA WHOLE BLOOD-Glycosylated hemoglobin (GHb) has been firmly established as an index of long-term blood glucose concentrations and as a measure of the risk for the development of complications in patients with diabetes mellitus. Formation of GHb is essentially irreversible, and the concentration in the blood depends on both the life span of the red blood cell (average 120 days) and the blood glucose concentration. Because the rate of formation of GHb is directly proportional to the concentration of glucose in the blood, the GHb concentration represents the integrated values for glucose over the preceding 6-8 weeks. Any condition that alters the life span of the red blood cells has the potential to alter the GHb level. Samples from patients with hemolytic anemias will exhibit decreased glycated hemoglobin values due to the shortened life span of the red cells. This effect will depend upon the severity of the anemia. Samples from patients with polycythemia or post-splenectomy may exhibit increased glycated hemoglobin values due to a somewhat longer life span of the red cells. Glycosylated hemoglobins results from patients with HbSS, HbCC, and HbSC and HbD must be interpreted with caution, given the pathological processes, including anemia, increased red cell turnover, transfusion requirements, that adversely impact HbA1c as a marker of long-term glycemic control. In these conditions, alternative forms of testing such as glycated serum protein (fructosamine) should be considered.

References

1. Tietz Textbook of Clinical Chemistry and Molecular Diagnostics, edited by Carl A Burtis, Edward R. Ashwood, David E. Bruns, 4th Edition, Elsevier publication, 2006, 879-884.
2. Forsham PH. Diabetes Mellitus: A rational plan for management. Postgrad Med 1982, 71, 139-154.
3. Mayer TK, Freedman ZR: Protein glycosylation in Diabetes Mellitus: A review of laboratory measurements and their clinical utility. Clin Chim Acta 1983, 127, 147-184.

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BIOCHEMISTRY**COMPLETE CARE ACTIVE WOMEN****GLUCOSE FASTING (FBS), FLUORIDE PLASMA**

FBS-FASTING BLOOD SUGAR(GLUCOSE)	70	(Normal <100, Impaired fasting glucose: 100 to 125, Diabetes mellitus: >=126 (on more than 1 occasion) (ADA guidelines 2024))
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METHOD : SPECTROPHOTOMETRY, HEXOKINASE

KIDNEY FUNCTION TEST, SERUM

BLOOD UREA NITROGEN	10	6 - 20	mg/dL
METHOD : SPECTROPHOTOMETRY, KINETIC TEST WITH UREASE AND GLUTAMATE DEHYDROGENASE			
CREATININE	0.77	0.60 - 1.10	mg/dL
METHOD : SPECTROPHOTOMETRY, KINETIC JAFFE			
BUN/CREAT RATIO	12.99	8.0 - 15.0	
METHOD : CALCULATED PARAMETER			
URIC ACID	6.5 High	2.4 - 5.7	mg/dL
METHOD : SPECTROPHOTOMETRY, BIURET			
TOTAL PROTEIN	7.1	6.0 - 8.0	g/dL
METHOD : BIURET			
ALBUMIN	4.3	3.97 - 4.94	g/dL
METHOD : SPECTROPHOTOMETRY, BROMOCRESOL GREEN (BCG) - DYE BINDING			
GLOBULIN	2.8	2.0 - 3.5	g/dL
METHOD : CALCULATED PARAMETER			
CALCIUM	9.3	8.6 - 10.0	mg/dL
METHOD : SPECTROPHOTOMETRY, NM - BAPTA			
SODIUM, SERUM	138	135 - 145	mmol/L
METHOD : ISE INDIRECT			
POTASSIUM, SERUM	3.89	3.5 - 5.3	mmol/L
METHOD : ISE INDIRECT			
CHLORIDE, SERUM	101	97 - 110	mmol/L
METHOD : ISE INDIRECT			

LIVER FUNCTION PROFILE, SERUM
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BILIRUBIN, TOTAL	0.30	Upto 1.2	mg/dL
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METHOD : COLORIMETRIC DIAZO

BILIRUBIN, DIRECT	0.15	< or = 0.3	mg/dL
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METHOD : COLORIMETRIC DIAZO

BILIRUBIN, INDIRECT	0.15	0.1 - 1.0	mg/dL
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METHOD : CALCULATED PARAMETER

TOTAL PROTEIN	7.1	6.0 - 8.0	g/dL
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METHOD : BIURET

ALBUMIN	4.3	3.97 - 4.94	g/dL
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METHOD : SPECTROPHOTOMETRIC - BROMOCRESOL GREEN (BCG)

GLOBULIN	2.8	2.0 - 3.5	g/dL
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METHOD : CALCULATED PARAMETER

ALBUMIN/GLOBULIN RATIO	1.5	1.0 - 2.1	RATIO
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METHOD : CALCULATED

ASPARTATE AMINOTRANSFERASE (AST/SGOT)	21	< OR = 35	U/L
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METHOD : SPECTROPHOTOMETRY, WITH PYRIDOXAL PHOSPHATE ACTIVATION-IFCC

ALANINE AMINOTRANSFERASE (ALT/SGPT)	22	< OR = 35	U/L
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METHOD : SPECTROPHOTOMETRY, WITH PYRIDOXAL PHOSPHATE ACTIVATION-IFCC

ALKALINE PHOSPHATASE	84	35 - 104	U/L
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METHOD : SPECTROPHOTOMETRY, PNPP, AMP BUFFER - IFCC

GAMMA GLUTAMYL TRANSFERASE (GGT)	10	0 - 40	U/L
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METHOD : ENZYMATIC COLORIMETRIC ASSAY STANDARDIZED AGAINST IFCC / SZASZ

LACTATE DEHYDROGENASE	136	125 - 220	U/L
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METHOD : SPECTROPHOTOMETRY, LACTATE TO PYRUVATE - UV-IFCC

PHOSPHORUS, SERUM

PHOSPHORUS	4.0	2.5 - 4.5	mg/dL
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METHOD : SPECTROPHOTOMETRY, METHYLTHYMOL BLUE

MAGNESIUM, SERUM

MAGNESIUM, SERUM	2.1	1.6 - 2.6	mg/dL
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METHOD : COLORIMETRIC ENDPOINT METHOD

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Results

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Interpretation(s)

GLUCOSE FASTING (FBS), FLUORIDE PLASMA-TEST DESCRIPTION

Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and so that no glucose is excreted in the urine.

Increased in: Diabetes mellitus, Cushing's syndrome (10 – 15%), chronic pancreatitis (30%). Drugs: corticosteroids, phenytoin, estrogen, thiazides.

Decreased in: Pancreatic islet cell disease with increased insulin, insulinoma, adrenocortical insufficiency, hypopituitarism, diffuse liver disease, malignancy (adrenocortical, stomach, fibrosarcoma), infant of a diabetic mother, enzyme deficiency diseases (e.g. galactosemia), Drugs-insulin, ethanol, propranolol, sulfonylureas, tolbutamide, and other oral hypoglycemic agents.

NOTE: While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values), there is wide fluctuation within individuals. Thus, glycosylated hemoglobin (HbA1c) levels are favored to monitor glycemic control.

High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment, Renal Glycosuria, Glycaemic index & response to food consumed, Alimentary Hypoglycemia, Increased insulin response & sensitivity etc.

Glucose fasting, plasma as per ADA Guideline

NORMAL RANGE: <100 mg/dl

PRE-DIABETIC RANGE: 100-125 mg/dl (Impaired Fasting Glucose)

DIABETIC RANGE: = 126 mg/dl

LIVER FUNCTION PROFILE, SERUM-Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Bilirubin is excreted in bile and urine, and elevated levels may give yellow discoloration in jaundice. **Elevated levels** results from increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease. Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of Hemolytic or pernicious anemia, Transfusion reaction & a common metabolic condition termed Gilbert syndrome, due to low levels of the enzyme that attaches sugar molecules to bilirubin.

AST is an enzyme found in various parts of the body. AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. AST levels may also increase after a heart attack or strenuous activity. ALT test measures the amount of this enzyme in the blood. ALT is found mainly in the liver, but also in smaller amounts in the kidneys, heart, muscles, and pancreas. It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. AST levels increase during acute hepatitis, sometimes due to a viral infection, ischemia to the liver, chronic hepatitis, obstruction of bile ducts, cirrhosis.

ALP is a protein found in almost all body tissues. Tissues with higher amounts of ALP include the liver, bile ducts and bone. Elevated ALP levels are seen in Biliary obstruction, Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Paget's disease, Rickets, Sarcoidosis etc. Lower-than-normal ALP levels seen in Hypophosphatasia, Malnutrition, Protein deficiency, Wilson's disease.

GGT is an enzyme found in cell membranes of many tissues mainly in the liver, kidney and pancreas. It is also found in other tissues including intestine, spleen, heart, brain and seminal vesicles. The highest concentration is in the kidney, but the liver is considered the source of normal enzyme activity. Serum GGT has been widely used as an index of liver dysfunction. Elevated serum GGT activity can be found in diseases of the liver, biliary system and pancreas. Conditions that increase serum GGT are obstructive liver disease, high alcohol consumption and use of enzyme-inducing drugs etc.

Total Protein also known as total protein, is a biochemical test for measuring the total amount of protein in serum. Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma, Waldenstrom's disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic syndrome, Protein-losing enteropathy etc.

Albumin is the most abundant protein in human blood plasma. It is produced in the liver. Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc.

MAGNESIUM, SERUM-Description- Magnesium is primarily an intracellular ion associated with GI absorption and renal excretion. Second most abundant ion in bone. It functions as co-factor in numerous enzymes e.g. ATPase. 65-70% of Mg is in ionized state and nearly 35% is protein bound.

Interpretation-

Increased in- Dehydration, Tissue trauma, Renal failure, Hypothyroidism, excessive intake of antacid.

Decrease in- Chronic diarrhea, Enteric fistula, Starvation, Chronic alcoholism, Total parenteral Nutrition, Diuretics.

Note- Hypomagnesemia is associated with weakness, tetany, disorientation and somnolence

Limitation-

- Hemolysis yields elevated levels of Mg being an intracellular ion.
- Serum magnesium levels may remain normal even when total body stores of magnesium are depleted up to 20%

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BIOCHEMISTRY - LIPID**COMPLETE CARE ACTIVE WOMEN****LIPID PROFILE WITH CALCULATED LDL,SERUM**

CHOLESTEROL, TOTAL	169	Desirable <200 Borderline: 200-239 High >240	mg/dL
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METHOD : ENZYMATIC COLORIMETRIC ASSAY

TRIGLYCERIDES	122	Normal: < 150 Borderline high: 150 - 199 High: 200 - 499 Very High: >= 500	mg/dL
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METHOD : ENZYMATIC COLORIMETRIC ASSAY

HDL CHOLESTEROL	40	Low: <40 High >=60	mg/dL
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METHOD : HOMOGENEOUS ENZYMATIC COLORIMETRIC ASSAY

CHOLESTEROL LDL	105	Optimal <100 Near Optimal 100-129 Borderline High 130-159 High 160-189 Very high >190	mg/dL
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METHOD : CALCULATED PARAMETER

NON HDL CHOLESTEROL	129	Optimal <130 Desirable 130-159 Borderline High 160-189 High 190-219 Very high >=220	mg/dL
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METHOD : CALCULATED PARAMETER

VERY LOW DENSITY LIPOPROTEIN	24.4	< /= 30	mg/dL
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METHOD : CALCULATED

CHOL/HDL RATIO	4.2	Optimal < 3.5 Desirable 3.5 - 5.0 High Risk > 5.0	
LDL/HDL RATIO	2.6	Desirable/Low Risk: 0.5 -3.0 Borderline/Moderate Risk: 3.1 - 6.0 High Risk: > 6.0	

METHOD : CALCULATED PARAMETER

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Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A. CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

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Punjab, India

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ULR No. 775000017592135-0080

PATIENT NAME : BHAVYA SHARMA

REF. DOCTOR : SELF

CODE/NAME & ADDRESS : C000125533

AGILUS DIAGNOSTICS PSC-MOHALI (HOME)
SCO NO-2, PHASE-6 S.A.S. NAGAR, RUPNAGAR
MOHALI 160055
9872331981

ACCESSION NO : 0080ZE000881

PATIENT ID : BHAVF04100027

CLNT. PATIENT ID: BHAVF04100027

ABHA NO :

AGE/SEX : 25 Years Female

DRAWN : 04/05/2026 09:32:26

RECEIVED : 04/05/2026 11:13:39

REPORTED : 04/05/2026 13:38:49

Test Report Status	Preliminary	Results	Biological Reference Interval	Units
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NEPHELOMETRY

COMPLETE CARE ACTIVE WOMEN

C-REACTIVE PROTEIN, SERUM (QUANTITATIVE)

C-REACTIVE PROTEIN	3.4	< 5.0	mg/L
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METHOD : PARTICLE ENHANCED IMMUNOTURBIDIMETRIC ASSAY

Interpretation(s)

C-REACTIVE PROTEIN, SERUM (QUANTITATIVE)- Test Description:

A CRP test measures the amount of CRP in the blood to detect inflammation due to acute conditions or to monitor the severity of disease in chronic conditions. CRP is one of the proteins commonly referred to as acute phase reactants. CRP is distinguished by its rapid response to trauma or infection. Synthesis of CRP increases within 4-6 hours of onset of inflammation, reaching peak values within 1-2 days. CRP levels also fall quickly after resolution of inflammation since its half life is 6 hours.

This standard CRP test is not to be confused with a hs-CRP test. These are two different tests that measure CRP and each test measures a different range of CRP levels in the blood for different purposes. The standard CRP test measures high levels of protein observed in diseases that cause significant inflammation.

Test Interpretation:

Increased CRP level: Increasing amount of CRP in the blood suggests the presence of inflammation but will not identify its location or the cause.

Suspected bacterial infection: a high CRP level can confirm that you have a serious bacterial infection.

Chronic inflammatory disease: high levels of CRP suggest a flare-up if you have a chronic inflammatory disease or that treatment has not been effective.

Testing for CRP is indicated in the following clinical situations - monitoring recovery from surgery, myocardial infarction, transplantation, inflammatory bowel disease, rheumatic diseases and infectious diseases. Measuring and charting C-reactive protein values can also prove useful in determining disease progress or the effectiveness of treatments

CRP levels can be elevated in the later stages of pregnancy as well as with the use of birth control pills or hormone replacement therapy (i.e., estrogen). Higher levels of CRP have also been observed in people who are obese. CRP can also be increased in people who have cancer.

Recommendation: The hs-CRP test precisely detects lower levels of the protein than that measured by the standard CRP test and is also used to evaluate individuals for risk of cardiovascular disease. It measures CRP in the range from 0.15 to 20 mg/L.

Limitation:

CRP levels in autoimmune diseases may show little or no increase unless infection is present. Levels may not increase in conditions like pregnancy, angina, seizures, asthma, common cold. The main limitation of CRP is in its non-specific response and should not be interpreted without a complete clinical history and evaluation.

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Test Report Status **Preliminary** **Results** **Biological Reference Interval** **Units**
ENDOCRINOLOGY**COMPLETE CARE ACTIVE WOMEN****THYROID PANEL II (FT3, FT4, TSH) SERUM**

FREE TRIIODOTHYRONINE (FT3)	2.59	2.0 - 4.4	pg/mL
METHOD : ECLIA			
FREE THYROXINE (FT4)	0.96	0.93 - 1.70	ng/dL
METHOD : ECLIA			
TSH (ULTRASENSITIVE)	3.630	Non Pregnant Women 0.27 - 4.20 Pregnant Women (As per American Thyroid Association) 1st Trimester 0.100 - 2.500 2nd Trimester 0.200 - 3.000 3rd Trimester 0.300 - 3.000	μIU/mL

METHOD : ECLIA

Interpretation(s)

Sr. No.	TSH	FT4	FT3	Possible Conditions
1	High	Low	Low	(1) Primary Hypothyroidism (2) Chronic autoimmune Thyroiditis (3) Post Thyroidectomy (4) Post Radio-Iodine treatment
2	High	Normal	Normal	(1) Subclinical Hypothyroidism (2) Patient with insufficient thyroid hormone replacement therapy (3) In cases of Autoimmune/Hashimoto thyroiditis (4). Isolated increase in TSH levels can be due to Subclinical inflammation, drugs like amphetamines, Iodine containing drug and dopamine antagonist e.g. domperidone and other physiological reasons.
3	Normal/Low	Low	Low	(1) Secondary and Tertiary Hypothyroidism
4	Low	High	High	(1) Primary Hyperthyroidism (Graves Disease) (2) Multinodular Goitre (3) Toxic Nodular Goitre (4) Thyroiditis (5) Over treatment of thyroid hormone (6) Drug effect e.g. Glucocorticoids, dopamine, T4 replacement therapy (7) First trimester of Pregnancy
5	Low	Normal	Normal	(1) Subclinical Hyperthyroidism
6	High	High	High	(1) TSH secreting pituitary adenoma (2) TRH secreting tumor

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7	Low	Low	Low	(1) Central Hypothyroidism (2) Euthyroid sick syndrome (3) Recent treatment for Hyperthyroidism
8	Normal/Low	Normal	High	(1) T3 thyrotoxicosis (2) Non-Thyroidal illness
9	Low	High	Normal	(1) T4 Ingestion (2) Thyroiditis (3) Interfering Anti TPO antibodies

REF: 1. TIETZ Fundamentals of Clinical chemistry 2. Guidelines of the American Thyroid association during pregnancy and Postpartum, 2011.

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SPECIALISED CHEMISTRY - ANEMIA**COMPLETE CARE ACTIVE WOMEN****IRON AND TIBC STUDIES, SERUM**

IRON	73	37 - 145	µg/dL
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METHOD : SPECTROPHOTOMETRY, FERROZINE

TOTAL IRON BINDING CAPACITY	308	250 - 400	µg/dL
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METHOD : CALCULATED PARAMETER

% SATURATION	24	15 - 50	%
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METHOD : SPECTROPHOTOMETRY, FERROZINE

Interpretation(s)

IRON AND TIBC STUDIES, SERUM - Total iron binding capacity (TIBC) measures the blood's capacity to bind iron with transferrin and thus is an indirect way of assessing transferrin level.

Taken together with serum iron and percent transferrin saturation this test is performed when there is a concern about anemia, iron deficiency or iron deficiency anemia. However, because the liver produces transferrin, alterations in liver function (such as cirrhosis, hepatitis, or liver failure) must be considered when performing this test.

Increased in:

- iron deficiency
- acute and chronic blood loss
- acute liver damage
- progesterone birth control pills

Decreased in:

- hemochromatosis
- cirrhosis of the liver
- thalassemia
- anemias of infection and chronic diseases
- nephrosis
- hyperthyroidism

The percent Transferrin saturation = Serum Iron/TIBC x 100

Unsaturated Binding Capacity (UIBC) = TIBC - Serum Iron.

Limitations: Estrogens and oral contraceptives increase TIBC and Asparaginase, chloramphenicol, corticotropin, cortisone and testosterone decrease the TIBC level.

Reference:

1. Tietz Textbook of Clinical Chemistry and Molecular Diagnostics, edited by Carl A Burtis, Edward R. Ashwood, David E. Bruns, 4th Edition, Elsevier publication, 2006, 563, 1314-1315.
2. Wallach's Interpretation of Diagnostic tests, 9th Edition, Ed Mary A Williamson and L Michael Snyder. Pub Lippincott Williams and Wilkins, 2011, 234-235.

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CLINICAL PATH - URINALYSIS**COMPLETE CARE ACTIVE WOMEN****PHYSICAL EXAMINATION, URINE**

COLOR	PALE YELLOW
APPEARANCE	SLIGHTLY HAZY

CHEMICAL EXAMINATION, URINE

PH	5.5	4.5 - 7.5
METHOD : REFLECTANCE SPECTROPHOTOMETRY- DOUBLE INDICATOR METHOD		
SPECIFIC GRAVITY	1.020	1.005 - 1.030
METHOD : REFLECTANCE SPECTROPHOTOMETRY (PKA CHANGE OF PRETREATED POLY ELECTROLYTES)		
PROTEIN	NOT DETECTED	NOT DETECTED
METHOD : REFLECTANCE SPECTROPHOTOMETRY (PROTEIN-ERROR-OF-INDICATORS PRINCIPLE)		
GLUCOSE	NOT DETECTED	NEGATIVE
METHOD : REFLECTANCE SPECTROPHOTOMETRY (GLUCOSE OXIDASE/PEROXIDASE METHOD)		
KETONES	NOT DETECTED	NOT DETECTED
METHOD : REFLECTANCE SPECTROPHOTOMETRY (SODIUM NITROPRUSSIDE REACTION)		
BLOOD	NOT DETECTED	NEGATIVE
METHOD : REFLECTANCE SPECTROPHOTOMETRY (PEROXIDASE METHOD)		
BILIRUBIN	NOT DETECTED	NOT DETECTED
METHOD : REFLECTANCE SPECTROPHOTOMETRY (DIAZO REACTION)		
UROBILINOGEN	NORMAL	NORMAL
METHOD : REFLECTANCE SPECTROPHOTOMETRY (EHRICH'S REACTION)		
NITRITE	NOT DETECTED	NOT DETECTED
METHOD : REFLECTANCE SPECTROPHOTOMETRY, CONVERSION OF NITRATE TO NITRITE		
LEUKOCYTE ESTERASE	DETECTED (+)	NOT DETECTED
METHOD : MICROSCOPIC EXAMINATION		

MICROSCOPIC EXAMINATION, URINE

RED BLOOD CELLS	NOT DETECTED	0 - 2	/HPF
METHOD : MICROSCOPIC EXAMINATION			
PUS CELL (WBCS)	10-15	0-5	/HPF
METHOD : MICROSCOPIC EXAMINATION			
EPITHELIAL CELLS	10-15	0-5	/HPF

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Test Report Status	Preliminary	Results	Biological Reference Interval	Units
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METHOD : MICROSCOPIC EXAMINATION

CASTS

NOT DETECTED

METHOD : MICROSCOPIC EXAMINATION

CRYSTALS

NOT DETECTED

BACTERIA

NOT DETECTED

NOT DETECTED

METHOD : MICROSCOPIC EXAMINATION

YEAST

NOT DETECTED

NOT DETECTED

METHOD : MICROSCOPIC EXAMINATION

Interpretation(s)

The following table describes the probable conditions, in which the analytes are present in urine

Presence of	Conditions
Proteins	Inflammation or immune illnesses
Pus (White Blood Cells)	Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment
Glucose	Diabetes or kidney disease
Ketones	Diabetic ketoacidosis (DKA), starvation or thirst
Urobilinogen	Liver disease such as hepatitis or cirrhosis
Blood	Renal or genital disorders/trauma
Bilirubin	Liver disease
Erythrocytes	Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases
Leukocytes	Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions
Epithelial cells	Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time
Granular Casts	Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein
Hyaline casts	Physical stress, fever, dehydration, acute congestive heart failure, renal diseases

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Calcium oxalate	Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice
Uric acid	arthritis
Bacteria	Urinary infection when present in significant numbers & with pus cells.
Trichomonas vaginalis	Vaginitis, cervicitis or salpingitis

Reference:

Henry's Clinical Diagnostics and Management by Laboratory Methods 21st edition (pg 410).

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SPECIALISED CHEMISTRY - HORMONE

PROLACTIN, SERUM

PROLACTIN	55.14 High	4.79 - 23.3	ng/mL
-----------	------------	-------------	-------

METHOD : ECLIA

Comments

ADV : FOR PROLACTIN REPEAT WITH POOLED SERUM SAMPLE, IF CLINICALLY INDICATED.

Interpretation(s)

Prolactin is a protein hormone secreted by anterior pituitary gland & placenta (in pregnancy). The secretion is regulated physiologically by inhibitory & releasing factors of hypothalamus. The major physiologic action of prolactin is the initiation & maintenance of lactation in women. Hyperprolactinemia inhibits gonadotrophin secretion & can produce hypogonadism in men & women.

The clinical use of prolactin levels is in the diagnosis & management of male & female hypogonadism. Increased levels seen in : 1. Pituitary tumour. 2. Hypothalamic lesions. 3. Hypothyroidism. 4. Antidepressants. 5. Stress.

NOTE : Various drugs & physiological factors can give rise to falsely elevated levels. Due to its episodic secretion, high prolactin values should be reconfirmed by performing the test on a pooled serum sample from specimens drawn at 6 to 20 minutes interval.

MALE: Hyperprolactinaemia in males may be associated with decreased libido, impotence, infertility, gynaecomastia.

FEMALE: Prolactin secretion from pituitary shows significant diurnal, episodic & cyclical variations. Following is a suggested approach to hyperprolactinaemia in females:

Prolactin Level	Interpretation	Remarks, Often associated with
25 - 50 ng/ml	Mild Prolactin excess	physiological conditions like stress, exercise, pregnancy, lactation etc. This may not be associated with clinical hyperprolactinaemia and needs review after a month.
51 - 75 ng/ml	Moderate Prolactin Excess	clinical hyperprolactinaemia - short luteal phase, oligomenorrhea
Above 100 ng/ml	Marked prolactin excess	clinical hyperprolactinaemia - hypogonadism, amenorrhea, galactorrhea
Above 200 ng/ml	Marked prolactin excess	pituitary adenoma requiring further workup.
References : 1. Diagnosis & Treatment of hyperprolactinaemia. The endocrine society clinical practice guideline, 2011 2. Diagnosis & Management of hyperprolactinemia. Canadian Medical Association CMAJ. Sept. 16, 2003; 169(6)		

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Test Report Status **Preliminary**

Results

Biological Reference Interval Units

SPECIALISED CHEMISTRY - TUMOR MARKER

COMPLETE CARE ACTIVE WOMEN

RESULT PENDING

CA 125 (OVARIAN CANCER MONITOR),SERUM

RESULT PENDING



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Test Report Status **Preliminary**

Results

Biological Reference Interval Units

SPECIALISED CHEMISTRY - VITAMIN

COMPLETE CARE ACTIVE WOMEN

25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM

25 - HYDROXYVITAMIN D

16.96 Low

Deficiency: ng/mL

< 20.0

Insufficiency:

20.0 - < 30.0

Sufficiency:

30.0 -100.0

Toxicity > 100.0

VITAMIN B12(CYANOCOBALAMINE), SERUM

VITAMIN B12

151.3 Low

197 - 771

pg/mL

Interpretation(s)

25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM- Test description

Vitamin D has anti-inflammatory and immune-modulating properties and it works towards the bones, teeth, intestines, immune system, pancreas, muscles and brain. It helps to maintain normal calcium and phosphate levels. Vitamin D is a fat-soluble vitamin. Also called as "Sunshine Vitamin". Two main forms as Cholecalciferol (vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight (Type B UV) exposure & Ergocalciferol (vitamin D2) present mainly in dietary sources.

Vit D25(OH)D deficiency is seen due to poor or inadequate sunlight exposure, Nutritional or dietary deficiency or fat malabsorption, Severe Hepatocellular disease, Secondary hyperparathyroidism, Hypocalcemia tetany which can cause involuntary contraction of muscles, leading to cramps and spasms, Rickets in children, Osteomalacia in adults- due to vitamin D deficiency mainly, Older adults- osteoporosis. (Increased risk of bone fractures) due to long-term effect of calcium and/or vitamin D deficiency, Other conditions that are precipitated by Vit D deficiency included increased cardiovascular risk, low immunity & chronic renal failure.

Elevated levels may be seen in patients taking supplements(hence recommended to repeat after 3 months for estimation of accurate levels), Vitamin D intoxication, sarcoidosis and malignancies containing non regulated 1-alpha hydroxylase in the lesion.

Recommendations

1.To prevent biotin interference the patient should be atleast 8 hours fasting before submitting the sample 2.25(OH)D is the analyte of choice for determination of the Vitamin D status as it is the major storage & active form of Vitamin D and has longer half-life. 3. Kidney Disease Outcomes Quality Initiatives (KDOQI) and Kidney Disease Improving Global Outcomes (KDIGO) recommend activated vitamin D testing for CKD patients.

Note-Our Vitamin D assays is standardized to be in alignment with the ID-LC/MS/MS 25(OH)vitamin D Reference Method Procedure (RMP), the reference procedure for the Vitamin D Standardization Program (VDSP). The VDSP, a collaboration of the National Institutes of Health Office of Dietary Supplements, National Institute of Technology and Standards, Centers for Disease Control and Ghent University, is an initiative to standardize 25(OH)vitamin D measurement across methods.

Reference: 1.Wallach Interpretation of diagnostic test, 10th edition.

VITAMIN B12(CYANOCOBALAMINE), SERUM-Test description

1.Measures the amount of Vitamin B12/ Cyanocobalamin or Methyl cobalamin in blood.2. Done in Anemic conditions like Megaloblastic anemia, pernicious anemia, dietary folate deficiencies,3.Workup of neuropathies especially due to diabetes.4.Nerve health and it is monitored in treatment of nerve damage.5.Importance vitamin for women of childbearing age and for older people.

1.Part of water-soluble B complex of vitamins. 2. It is essential in DNA synthesis, hematopoiesis & CNS integrity.3.Source for B12 is dietary foods like milk, yoghurt, eggs, meat, fortified cereals, bread. 4.Absorption depends on the HCl secreted by the stomach and occurs in intestines. 5. It is part of enterohepatic circulation, hence excreted in feces(approx. 0.1% per day)

Test interpretation

Higher than normal levels are in patients on Vitamin supplements or patients with COPD, CRF, Diabetes, Liver cell damage, Obesity, Polycythemia.

Decreased levels seen in

 DR.CHANDNI GARG
 CONSULTANT PATHOLOGIST

 Dr.Pranjali Vasisht
 LAB HEAD

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View Report

PERFORMED AT :

Agilus Diagnostics Ltd

29 Sco, Sector 11 D

Chandigarh, 160011

Punjab, India

Tel : 9111591115, Fax :

CIN - U74899PB1995PLC045956



ULR No.775000017592135-0080

**PATIENT NAME : BHAVYA SHARMA****REF. DOCTOR : SELF****CODE/NAME & ADDRESS : C000125533**
 AGILUS DIAGNOSTICS PSC-MOHALI (HOME)
 SCO NO-2, PHASE-6 S.A.S. NAGAR, RUPNAGAR
 MOHALI 160055
 9872331981
ACCESSION NO : 0080ZE000881**PATIENT ID : BHAVF04100027**

CLNT.PATIENT ID: BHAVF04100027

ABHA NO :**AGE/SEX : 25 Years Female****DRAWN : 04/05/2026 09:32:26****RECEIVED : 04/05/2026 11:13:39****REPORTED : 04/05/2026 13:38:49**

Test Report Status	Preliminary	Results	Biological Reference Interval	Units
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Inflammatory bowel disease, Pernicious anemia - genetic deficiency of intrinsic factor - necessary for Vit B12 absorption, Strict vegetarians lead to sub-clinical B12 deficiency- high among elderly patients, Malabsorption due to gastrectomy, smoking, pregnancy, multiple myeloma & hemodialysis, Alcohol & drugs like amino salicylic acid, anticonvulsants, cholestyramine, cimetidine, Hyperthyroidism (High levels of thyroid), Seen in mothers of children with (NTD) Neural tube defects- hence fortification and supplements are advised in expecting mothers

Recommendations-1.To prevent biotin interference the patient should be at least 8 hours fasting before submitting the sample. 2. Vit B12 and Folic acid evaluated together in macrocytic anemias to avoid methyl folate trap. Carmel's composite criteria for inadequate Vit B12 status: Serum vitamin B12 < 148 pmol/L, or 148-258 pmol/L and MMA > 0.30µmol/L, or tHcy > 13 nmol/L (females) and >15 nmol/L (males).

Associated Test-Holo-TC: Marker of vitamin B12 status -specificity and sensitivity better than serum vitamin B12, hence recommended in borderline and deficient cases for confirmation.

References-O-Leary F, Samman S. Vitamin B12 in health and disease. Nutrients. 2010 Mar 2(3):299-316.

****End Of Report****Please visit www.agilusdiagnostics.com for related Test Information for this accession**CONDITIONS OF LABORATORY TESTING & REPORTING**

1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
 - i. Specimen received is insufficient or inappropriate
 - ii. Specimen quality is unsatisfactory
 - iii. Incorrect specimen type
 - iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

Agilus Diagnostics Ltd
 Fortis Hospital, Sector 62, Phase VIII,
 Mohali 160062

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